

A Small, Low-Cost First Test of an Idea to Help Rats Learn

Introduction in simple words · Researcher: Solano Moreira Furlan

What is the idea?

Scientists found that a natural substance in the fluid around young brains, called FGF17, seems to help old mice remember things better. Another substance, called 7,8-DHF, is thought to help brain cells make stronger connections with each other. We want to know: if we give both, and also give the animals a rich, stimulating place to learn, do they become better at solving different kinds of problems than normal rats of the same age?

What will we do?

This is a deliberately small version. We use 30 healthy young rats in three groups of 10: normal rats; rats that spend time in the learning environment; and rats that get the two substances and the learning environment. Another 18 rats are helpers (12 to check that FGF17 really does something in the rat brain, 6 for practice and safety checks), and up to 12 more are held in reserve for one repeat of that check. The 30 main rats do the same three tasks (we expect to lose some along the way, so only about 18 to 21 may finish) — a puzzle where the rules suddenly change, a maze with new problems, and a swimming-pool memory test. Two halves of the group take the tasks in a different order at the same time, to save weeks. The check on FGF17 comes first: if there is no sign that it works, we stop and save the money. Some scientists doubt that 7,8-DHF works the way people first thought, so that is an open question. The rats get a small operation that places a thin tube into the brain's fluid space, daily injections for five weeks, limited food for some weeks and swimming tests, and are humanely put to sleep at the end; an animal-ethics committee must approve everything first.

Who does the work?

A good university laboratory in Brazil runs the experiment, following this document and detailed lab procedures that the researcher approves. The researcher guides it from a distance and keeps the secret group codes, so the staff giving the medicines do not know which rat got what, and checks the results independently.

How much will it cost?

Materials only (animals, chemicals, equipment parts, supplies) are planned at about USD 11,200. It could be as low as 8,000 (if the lab already owns the apparatus) or as high as 13,300, and up to about 14,800 if an import tax exemption is refused; the researcher's ceiling for materials is USD 12,000, and a reserve for currency swings would push the base case slightly above it. Staff pay and the university's running-cost charges are NOT included. The real total might be about USD 25,000 to 55,000 (an unverified estimate); the lab must quote it.

What will this test tell us — and not tell us?

It can tell us whether FGF17 shows a chemical sign of working in the rat brain, whether the lab can run all the tasks smoothly, and whether the results lean in a promising direction. With so few rats only a very large effect would show. Comparing the third group with the second shows what the substances add on top of the environment, but the substances are not tested alone. It cannot prove the idea works or say anything about people.

What if it goes well?

Then we plan a bigger experiment: first separating what each ingredient does, then using optogenetics (a technique that switches chosen brain cells on and off with light) to see how brain activity interacts with these substances.

The long-term dream: brain longevity

The original finding was in old mice: young brain fluid helped their memory. With ageing, and in diseases such as Alzheimer's, the bridges between brain cells and their insulation wear away, fewer new cells form, and the brain's tightly linked local "small-world" clusters and its rhythmic electrical pulses lose their organisation. The long-term hope is brain longevity: to test whether a treatment like this could help keep those bridges and their insulation, support new cells where the adult brain makes them, and preserve the brain's rhythm architecture into old age, so that thinking and memory decline less; and, in the same spirit, help people with learning difficulties or problems with how brain cells connect. That is a hope, not a result. This study uses healthy young rats and measures none of this (no connections, no new cells, no brain rhythms, no ageing, no disease). The next steps would be old rats and disease models with those measurements, then safety work, and only much later people; many promising brain treatments have failed on that road, and getting these substances into a human brain safely is a big obstacle. The rest of this document gives the technical details.

Technical protocol (Brazil mini proof of concept; core materials ≈ USD 11.2k, range 8.0-13.3k)

Does a candidate plasticity-promoting treatment (FGF17 + 7,8-DHF) combined with a structured learning environment raise performance across different cognitive tasks in rats? A minimal, staged proof-of-concept study run by a Brazilian host laboratory under remote oversight.

Researcher / sponsor	Solano Moreira Furlan (remote; the host laboratory runs the experiment)
Document	Brazil mini proof-of-concept protocol, version 3. The task methods, statistics logic and many safeguards come from a larger design reviewed in eight rounds in its India form and one round in its Brazilian full-size form; this mini design is reviewed in the rounds listed in section 11.
Date	9 October 2026
Status	Not approved by an ethics committee (CEUA). Brazil-specific rules and prices come from search snippets and reviewer knowledge and must be confirmed locally. Prices are estimates, not quotes. “crf” in the request was read as CSF (assumption to confirm).

Executive summary (technical)

Question. Does a candidate plasticity-promoting treatment — FGF17 (identified in young cerebrospinal fluid; Iram 2022 [1]) plus 7,8-DHF (a putative TrkB/BDNF-pathway agonist whose mechanism is disputed [2–4]) — combined with a structured enriched learning environment (ELE) raise the performance of healthy young rats across three tasks, one of which (set-shifting) uses stimulus dimensions not practised in the ELE, compared with untreated peers?

Design. One wave. Three arms of 10 enrolled rats (30 main rats; 6–7 expected completers per arm): C (vehicle, standard housing), E (vehicle + ELE) and D+E (FGF17 + 7,8-DHF + ELE). There is no drug-only arm, so drug and environment cannot be separated; the study compares the combination with enrichment alone and with untreated peers. 48 rats in total (30 main, 12 FGF17-engagement satellites, 6 practice/tolerability), plus 12 contingency rats registered as a maximum (60 at most; 56 cannulated). Phase 0a (reagent activity and engagement; a gate) precedes Phase 0b (behaviour). The two halves of the cohort (15 rats each, balanced by arm) run the three tasks in parallel and in different orders, which shortens the calendar to about 8 weeks. The study informs, and does not confirm, whether a larger study is justified.

Budget (honest). Materials-only core ≈ USD 11.2k base; ≈ 8.0k optimistic (host already owns the arenas, mazes and animal facility rates); ≈ 13.3k pessimistic; ≈ 14.3–14.8k if the CNPq import exemption is also refused. The base is inside the researcher-approved USD 12k materials ceiling only before a 10–15% currency reserve (12.3–12.9k with it); the pessimistic cases exceed it. Personnel, service fees, institutional overhead, equipment fees and a currency reserve are not included and must be quoted by the host; the all-in cost is probably USD 25–55k (unverified). The first-draft cost of USD 7.3k grew to 11.2k once arenas, mazes, cannulas, two imports and small consumables were itemised.

1. Aim, hypotheses and what the data can claim

Primary aim (feasibility): show that the reagent is active and engages its pathway in vivo; show that a Brazilian host can run the ELE and all three tasks with an acceptable completion rate; estimate variance and the direction of the D+E – C contrast.

Exploratory hypothesis: D+E performs better than control peers (C) and than enrichment alone (E).

Transfer: set-shifting is the primary transfer readout (odour and digging-medium dimensions are excluded from the ELE). Hebb-Williams and the water maze are spatial; the ELE has tunnels, climbing and wheels, so they are flagged as spatial, not transfer.

Allowed claims: feasibility, attrition, variance, direction with a 95% interval of D+E – C and D+E – E (overall index; set-shifting reported separately), and pathway engagement (pERK at the ventricular interface) — for the drug package as a whole, in combination with the ELE.

Not allowed: “smarter”, “general intelligence” or a g factor [8,9]; neuroplasticity; single-agent claims; any claim about the drug alone (there is no drug-only arm; D+E – E estimates the drug package on top of enrichment); TrkB-specific claims (pERK shows pathway engagement only); human, ageing or disease claims; any claim about network organisation, brain rhythms or new neurons; replication of Iram 2022 (an aged-mouse rescue; this asks about enhancement in healthy young adults, which no study found has tested).

2. Long-term goal, next experiment (including optogenetics) and translational roadmap

Recommended wording (translational reviewer, adopted):

“Our long-term aim (brain longevity) is to test whether combining a putatively plasticity-promoting biological treatment (an FGF17-type factor plus a TrkB/BDNF-pathway agent whose mechanism is validated; 7,8-DHF currently is not) with structured enriched learning can produce measurable cognitive benefit in humans. We pursue two **separate** programmes with distinct biology, models, ages and ethics: (a) neurodevelopmental conditions with altered connectivity (learning disabilities), and (b) age-related cognitive decline and early Alzheimer’s disease. We hypothesise that supporting synaptic and myelin plasticity is **one possible contributor** to such benefit; this is a hypothesis to be demonstrated with structural and functional evidence, not an assumption, and it is not addressed by the present study. Later stages would also ask whether such treatment preserves long-range and local (small-world) network organisation, supports new-cell formation where it occurs in the adult brain (adult neurogenesis is itself debated, especially in humans, and the FGF17 evidence concerns oligodendrocyte-lineage cells and myelination, not neurons) and preserves oscillatory rhythms (for example hippocampal theta and gamma, measured by local field potentials or EEG) in ageing; none of these is measured here. Prior neurotrophin programmes in Alzheimer’s have not shown benefit (including a direct-delivery AAV-NGF trial), brain delivery of protein factors is a major obstacle, and both candidate agents face unresolved questions (human CNS delivery of FGF17; whether 7,8-DHF engages TrkB). Each stage proceeds only on pre-registered target-engagement, structural and safety gates.”

Stage	Content	Gate to proceed
0 – this study	0a engagement/reagents → 0b behaviour in healthy young rats (mini proof of concept; tissue banked, histology not included).	Section 8.
1a – next experiment: powered rat confirmation	Both sexes; full factorial separating FGF17, 7,8-DHF and enrichment (and exercise); sustained infusion by pump; dose-response; durable vs on-drug effects; TrkB and FGFR necessity tests; synaptic readouts (PSD-95/synaptophysin on banked tissue, LTP).	Replicated effect on a pre-registered primary; structural effect.

Stage	Content	Gate to proceed
1b – next experiment: optogenetics (only if 1a is positive; mechanistic, not translational)	Question: how does controlled neuronal activity interact with FGF17 and with 7,8-DHF-driven plasticity? Candidate designs (to be designed and reviewed in their own round): (a) activity × plasticity-opening — optogenetic stimulation of a defined circuit (e.g. CaMKIIa-driven channelrhodopsin in prefrontal or hippocampal pyramidal neurons) during or after learning, with and without FGF17/7,8-DHF, asking whether activity amplifies oligodendrocyte-precursor proliferation, myelination and synaptic plasticity (mouse precedent: premotor-cortex stimulation, Gibson 2014 [10]; extension to prefrontal/hippocampal circuits is an extrapolation); (b) necessity — inhibitory opsins during learning or consolidation; (c) cell-type targeting of oligodendrocyte-lineage cells needs Cre lines that are mostly mouse (species switch or an AAV-based rat strategy). Requirements: AAV vectors with institutional biosafety approval (CIBio/CQB; classes under CTNBio RN 18/2018, class 1 approved by the CIBio and higher classes by CTNBio, under Lei 11.105/2005; the facility's CQB must cover genetically modified animals at the right containment level), fibre implants, calibrated light sources, opsin-negative/fluorophore-only and light-only controls (heating, visual artefacts), histological verification of opsin and fibre placement, and monitoring for stimulation-induced seizures/kindling. Beyond this budget; own funding, equipment, expertise and ethics approval.	1a positive; opsin tools validated in the host lab.
2 – disease models (separate tracks)	Aged rodents (Iram-type rescue); Alzheimer's models (5xFAD, APP/PS1, TgF344-AD) — amyloid-model rescue has repeatedly failed to translate; neurodevelopmental models at appropriate ages: Fmr1 knockout (fragile X) and Down-syndrome models (Ts65Dn is being superseded) as separate tracks.	Benefit with engagement and structural evidence.
3 – drug development	Clinically deliverable form (intranasal, intrathecal or engineered/receptor-selective agent — unproven); a validated TrkB-pathway drug with human-relevant PK; toxicology in a relevant species (one for a protein biologic under ICH S6(R1); more for a small molecule under ICH M3(R2)); FGF-class risks (mitogenesis/tumour promotion, angiogenesis); regulatory plan (ANVISA clinical-development dossier, DDCM under RDC 945/2024; ethics review within the national human-research ethics system, Lei 14.874/2024 as regulated by Decreto 12.651/2025; US IND / EU CTA; an engineered or gene-delivered FGF17 would also fall under ANVISA advanced-therapy rules and CTNBio).	Delivery solved; clean toxicology.
4 – human studies	Phase 1 safety/PK; biomarkers (MRI myelin water fraction/DTI, EEG, cognitive tests); structured learning input as digital training; Phase 2 in MCI/early AD and, separately, in neurodevelopmental populations (ANVISA and ethics-system scrutiny; assent plus proxy consent; independent data-safety monitoring).	Safety and biomarker signal.

Hard barriers that must be stated: no validated way exists to deliver FGF17 to the human brain; the mitogenic and angiogenic risk of an FGF-class CNS agent may never clear a paediatric risk-benefit assessment for a non-fatal indication such as a learning disability, and enhancement in people without disease carries a higher ethical bar than treatment of disease; 7,8-DHF is reported not to activate TrkB reliably in cell assays [2–4]; no human trials of FGF17 or 7,8-DHF were found (verify); healthy-animal enhancement may not transfer to disease or humans. The FGF23/phosphate effect is not the relevant class risk (a different, endocrine FGF).

What this study must deliver to earn Stage 1a: reagent activity and FGF17 pathway engagement (0a); a positive direction on the overall index (0b), with set-shifting reported separately; and, in the larger study, an Olig2/MBP histology direction on banked tissue. Covariation of structure with behaviour cannot be judged at this n.

3. Design and animals

Young adult male Wistar rats (about 8–10 weeks old, ≈ 250 – 300 g at surgery) from a CONCEA-registered animal facility (a university central biotério or institutional breeder; strain and sanitary status stated; SPF Wistar are bred in Brazil, for example at CEMIB/Unicamp, but no current price was found there, so supply, age-matched litters and price are confirmed directly with the facility). Males only. Housed on a reversed 12:12 light cycle so that dark-phase work falls in working hours, in perforated-divider cages in pairs of the same arm and time block (social contact with individual rations and less headcap chewing; single housing only with CEUA justification, identically in all arms). The sponsor randomises by baseline weight and source litter and holds the allocation and drug-code key (section 10). The 30 main rats are split into two halves (A and B, 5 per arm each) that take the tasks in different orders, and into five fixed time blocks of six rats (two per arm, assigned at random within arm and kept for the whole study; each cage pair is one A rat and one B rat of the same arm and block). Blinding: an unblinded host technician (with a named backup) who does no testing or scoring prepares coded vials and syringes from the sponsor's key, wrapped in opaque sleeves with opaque-hub needles (7,8-DHF is a yellow flavone, so a clear vehicle or a clear syringe would reveal the arm); the injector and weigher never test or score; surgeon and behaviour operators are blind to drug; welfare and endpoint decisions are made by staff blind to arm. The environment arm cannot be fully blinded, so ELE handlers and test-day scorers are different people, set-shifting and Hebb-Williams are scored live by testers blind to environment where possible and verified from video under coded IDs, and 20% of videos are re-scored for inter-rater agreement (ICC). Arena identity is rotated by day within each arena type so that it is not aliased with arm.

Group	Content	Enrolled
Main cohort (one wave)	3 arms $\times$ 10: C (vehicle + standard), E (vehicle + ELE), D+E (FGF17 + 7,8-DHF + ELE); halves A/B of 5 per arm	30
Engagement satellite – FGF17	At 30 min after one ICV bolus ($0.5 \mu\text{g}$ in $3 \mu\text{l}$, as in the main cohort): time-matched vehicle 4, FGF17 4, heat-inactivated FGF17 (boiled aliquot, equal mass) 4; restraint duration logged; same tissue method in all groups; t_0 = injector removal; punch of the periventricular wall (right hemisphere) and hippocampus taken first, then the remaining slab and cannula track photographed (order recorded); pERK/total ERK; a satellite with confirmed misplacement is excluded from the gate ratio and replaced from the contingency rats	12
Practice/tolerability	2 cannula placement checks; 4 vehicle-tolerability rats (2 low-DMSO solution, 2 CMC-Na suspension, each receiving 7,8-DHF in its vehicle for the full 35 days; weekly weights, daily abdominal-wall palpation score, necropsy scoring). With 2 rats per vehicle this is a screen for gross intolerance only: $>10\%$ weight loss or any palpation/necropsy sign of peritonitis or adhesions in either rat triggers a host and sponsor review and a switch of vehicle. The chosen vehicle is a 35-day exposure in all arms, so IP sites and abdominal pain are also scored daily in the main cohort.	6
Total	Cannulated rats: $30 + 12 + 2 = 44$	48
Contingency (registered maximum, not enrolled unless needed)	One FGF17 re-test (4 vehicle + 4 FGF17 + 4 heat-inactivated, section 8) $\rightarrow$ registered maximum 60 rats, 56 cannulated	12

Enrolment is 10 per arm because ICV-cannula attrition in rats is commonly 30–40% (a planning assumption, not a sourced rate); the plan assumes 60–70% completion (6–7 per arm). Cannula loss is logged by arm. Because climbing and wheels exist only in the enrichment arms, cap loss is expected to be higher there: protected recessed caps with a smooth collar, climbing structures ≥ 15 cm below the lid and solid-surface (rung-free) wheels are used, an arm-stratified tipping-point analysis is pre-specified, the delivered FGF17 dose is reported per D+E completer, a completion difference of ≥ 3 completers between arms is flagged and reported (this will happen in about 45% of runs even with equal true completion, so it is descriptive only), and a difference of ≥ 5 triggers a review and a possible stop (about 8% false alarm under equal completion; because cap loss is expected to be higher in the ELE arms, some gap is expected rather than a pure false alarm). Singleton rats whose cage-mate was lost stay in their divider cage with the empty side open; days single-housed are logged by arm and used as a sensitivity covariate, and the CEUA justification for single housing covers this case. Welfare: the combined burden (cannula for ≈ 7 weeks, 35 daily injections, food restriction, swimming) is expected to be graded Moderado (G2) under CONCEA RN 55/2022 Annex II (or GI3 on CEUA forms that still use the older GI1–GI4 scale), and the CEUA may grade the cumulative burden higher (G3 or G4, “grave” in Annex II; the exact label is to be confirmed); the narrative gives a cumulative-severity justification and the refinements, with a scoresheet and humane endpoints: $>15\%$ weight loss from the growth-adjusted target, injection-site lesions, cannula infection, hypothermia rule, neurological signs after ICV (any seizure: immediate euthanasia; circling or head tilt lasting >24 h: euthanasia), scoring daily and twice daily after surgery and during the restriction ramp, and a named growth curve (the supplying facility's Wistar growth chart) for the growth-adjusted target, and the headcap (loosened with intact margins: one re-cementing under isoflurane; detached or exposed bone:

euthanasia; decided by a veterinarian blind to arm, with the event logged by arm). Surgical, re-capping and terminal anaesthesia use isoflurane. The analgesia regimen is written by the host veterinarian (typically pre-emptive meloxicam daily for at least 72 h, dipyrone, and local lidocaine/bupivacaine at the incision, with antibiotic prophylaxis, eye lubricant, thermal support and fluids per SOP; post-operative scoring twice daily for 3 days, then daily); an opioid (buprenorphine or tramadol) is added if the CEUA requires it or pain scores warrant it. Tramadol, buprenorphine, other opioids and ketamine are controlled in Brazil and are used only through the host's authorised route (section 9). Surgery is limited to 10 rats a day on two frames with two surgeons (about 8 h each). Pre-specified exclusions: dye/histology-confirmed ventricular placement, humane endpoints, swim speed <50% of cohort median, failure to reach the set-shift training criterion.

Power (honest). With 6–7 completers per arm, a pairwise comparison reaches 80% power only for $d \approx 1.6$ – 1.8 (unadjusted). Healthy young rats are near ceiling on simple tasks, and a cross-species enrichment meta-analysis reports $g \approx 0.44$ in rodents (an unpublished dissertation [15]), so a null result is likely even if a modest true effect exists, and only a very large effect would be seen. Pilot effect sizes must not be used to power the next experiment from the point estimate [12,13].

4. Interventions

Component	Specification
FGF17 (days 0–6)	Recombinant mouse FGF17, one 50 µg pack (mouse-rat sequence alignment documented before ordering). Chronic guide cannula to the right lateral ventricle (scaled coordinates; a microinjection pump and Hamilton syringes are required). Four boluses on days 0, 2, 4, 6 in the D+E arm: 3 µl of ≈167 µg/ml (0.5 µg each; 2.0 µg per rat) in aCSF + 0.1% BSA (standard anti-adsorption practice, not a vendor specification) + heparin 10 µg/ml, awake, 0.5–1 µl/min with the injector left 2–3 min; vehicle boluses in C and E. The 167 µg/ml working stock lies within vendor reconstitution ranges (typically ≥100 µg/ml, up to 1 mg/ml; reconstituted FGF17 keeps 2–7 days at 4 °C), so aliquots are frozen at –80 °C in carrier and used within about 4 months, recovery after reconstitution and freeze-thaw and a pERK potency recheck are done before the main cohort, low-bind tubes are used, the solution is inspected for precipitate, and if unstable the concentration is lowered and the volume raised (up to 5 µl; the shortfall in delivered mass is then declared); dead volume is measured and primed so that 3 µl is delivered. Supply from the single pack: D+E 10 × 2.0 = 20 µg; satellites 4 × 0.5 + heat-inactivated 4 × 0.5 = 4 µg; contingency re-test 4 × 0.5 + 4 × 0.5 = 4 µg; bioassay ≈5 µg (generous; plus the boiled aliquot, below); freeze-thaw recovery loss and dead volume ≈10 µg; potency recheck ≈5 µg: ≈48 µg of 50 µg, so there is no margin and a 25 µg vial is the contingency. The pack is reconstituted in two lots; the working aliquots are used within about 4 months of reconstitution, the potency recheck is repeated within 2 weeks of day 0, and a re-test (which adds 5–6 weeks) triggers the contingency vial. The boiled (heat-inactivated) aliquot is itself bioassayed (<10% residual pERK activity) and checked for aggregation. <u>Delivery gap and dose anchor</u> : Iram 2022 infused continuously for about a week with an Alzet 1007D (≈84 µl delivered); one reviewer retrieved the Methods as recombinant mouse FGF17 at 25 µg/ml (≈2.1 µg continuous) plus a separate acute 0.5 µg bolus in 2 µl; another reviewer could not retrieve it, so the host must confirm it in the Methods (PMC9377328 or the UCL copy) before ordering. Four 0.5 µg boluses (2.0 µg total, 12 µl) approximately match that one reported total mass but not the exposure time: they are pulsatile and ≈7× smaller in volume, so a null remains confounded with delivery and FGF17 stays exploratory.
7,8-DHF (days 0–34)	5 mg/kg IP daily, days 0–34 (35 injections; rotated sites) in the D+E arm; vehicle IP in C and E. Daily order: testing (if scheduled) → 7,8-DHF injection → ELE/control session 30 min later, so the agent is on board during enrichment (mouse half-life ≈2 h) and the next day's testing falls at trough (≈18–24 h after dosing in mice; one oral rat report gives a terminal half-life of ≈23 h — if that is right the drug accumulates and testing is not drug-free). Rat precedent for 5 mg/kg IP daily includes 5 weeks in a rotenone model in 9-month-old rats [17] and an optic-nerve model with outcomes at 7–21 days [18]; mouse work exists [5]. The 35-day duration is at the long end of that precedent. Vehicle (decided by the tolerability check before the main cohort is ordered; the check is a screen only) : default is a low-DMSO solution (5% DMSO / 40% PEG300 / 5% Tween-80 / 50% water per Selleck, or 10% / 40% / 5% / 45% saline per MedChemExpress/InvivoChem, ≈2.5 mg/ml; prepared fresh); fallback is an aqueous CMC-Na suspension (no peer-reviewed intraperitoneal PK found; an IP suspension may give erratic absorption, so a null could reflect exposure). Oil vehicles are avoided. Necropsy scoring of peritoneal adhesions/granulomas in all main-cohort rats; identical vehicle in all arms.
Enriched learning environment (ELE)	Days 0–34, one 60-min solo session per day (the test rat alone; no partner rats, which removes social contact as a confound and the housing and handling of partner animals) 30 min after the 7,8-DHF injection, in the dark phase under red light [6,7]. This is a weak enrichment dose and a non-social one (declared). Minimum specification the host must meet regardless of local habit: arena ≥1 m ² (the same size for control), ≥3 novel objects per session, running wheels (solid surface) and tunnels/climbing in each ELE arena (arena identity rotated by day within type, so that it is not aliased with arm), ≥3 puzzle devices on a six-level ladder (latch, string-pull, lever, chew), small fixed-calorie rewards (counted per rat; deducted from the ration only once food restriction has begun), engagement minutes and ladder level logged per rat, an object library of ≥30 objects on a rotation schedule, and a fourth ELE operator for cleaning and resets. On the first day of a rat's set-shift tests the ELE/control session is replaced by a home-cage interval in all arms (dosing continues), so that the attentional set between test day 1 and day 2 is not differentially exposed to novel objects; that one lost session per rat is declared. <u>Excluded from the ELE: odour discrimination and digging puzzles</u> . Control arena: same duration and handling, empty. SOP with photos completed by the host before CEUA submission.

5. Timeline and workload (day 0 = first bolus, a Thursday; one wave)

Days	Activity
–22 to –16	Arrival (a Wednesday) and 7-day acclimatisation; reversed light cycle.
–15 to –13	Cannula surgery (Wed–Fri) with analgesia, ≤ 10 rats/day on two frames with two surgeons; day –12 (Saturday) is a reserve surgery day; ≥ 12 days recovery before day 0.
–12 to –1	Recovery, handling, weighing; arena habituation from day –7 in an empty control-type arena (all rats; arm-neutral).
0–6	FGF17/vehicle boluses (days 0, 2, 4, 6; Thu, Sat, Mon, Wed).
0–34	ELE/control sessions and 7,8-DHF/vehicle every day (35 doses), weekends included.
5–10	Gradual food restriction to 90–95% of the growth-adjusted target by day 10 (before task habituation begins on day 11); held through the evening of day 28 (humane limit $<85\%$); each rat is rationed individually (divider cages) 2 h after the end of its own ELE/control session. Rewards in the ELE are counted per rat. Hours since feeding at testing are recorded descriptively.
11–28	Tasks 1 and 2 run in parallel in two halves (A and B, 5 rats per arm each), in opposite orders. <u>Half A:</u> set-shifting days 11–22 (habituation 11–13; training days 14–18; tests start for 5 rats on each of days 19, 20 and 21 (start days are allocated by block in a Latin rotation: each block's three half-A rats, one per arm, start on three different days, so each day has five starters, one per block, and no block has more than two rats testing on one day) and last two consecutive days each: day 1 simple and compound discrimination and ID1, day 2 ID2 and ED; last finish day 22), then Hebb-Williams days 23–28 (adaptation 23–24, then 4 problems on days 25–28). <u>Half B:</u> Hebb-Williams days 11–16 (adaptation 11–12, 4 problems on days 13–16), then set-shifting days 17–28 (habituation 17–19, training 20–24, tests start on days 25, 26, 27 with the same block rotation; last finish day 28). Set-shift rules: the training criterion is 6 consecutive correct on each of a simple odour and a simple medium discrimination (training exemplars distinct from test exemplars); rats that reach it hold on one maintenance session a day; a rat that has not reached it by the end of its training window (day 18 for A, day 24 for B) is excluded from set-shifting; a day-before-test reminder is 4 trials with training exemplars; each arm gets 2, 2 and 1 starters over the three start days and the start day is recorded. Tests have a 2.25-h cap per rat per day and trials a 3-min limit and a 60-trial cap per stage. A stage unfinished at the day-1 cap resumes at the start of day 2; any carry-over stage and ID2 share the first 60 min of day 2, so that ED always keeps a protected budget of at least 75 min of the 2.25-h cap. A rat that fails a pre-ED stage (no criterion in 60 trials, or not completed within the day-2 cap) is a stage failure: it is excluded from the set-shifting analysis, flagged by arm, and counted as the worst rank in the rank/censored sensitivity analysis. If ED is not reached within 60 trials the score is the cap (60). If the protected ED budget ends before 60 trials, the ED observation is time-censored: it is assigned the cap (60) in the index and in the fallback primary, and it is treated as right-censored at the trials completed in the set-shifting-alone censored (Cox) analysis and in the ED > ID2 validity check; time-censoring and stage times are reported per rat and by arm. There is no day 3, so Hebb-Williams or the water maze never overlaps an unfinished test. The overnight gap between day 1 and day 2 is a declared deviation from single-session protocols. Peak: 10 rats in set-shift testing in one day, but at most about 4 testing at the same moment; at most 5 digging boxes per ventilated room (two rooms) to limit odour cross-contamination, with half-B training sequenced so that testing and training never need more than 10 boxes at once. Food restriction ends on the evening of day 28.
29–34	Task 3 (last; non-appetitive): water maze for all 30 rats, run one block at a time (a block's six rats swim interleaved, 4 trials each, in about 40 min, inside their pre-dose window) — acquisition days 29–32, probe day 33, visible-platform test day 34 (Appendix A). It runs through one weekend (two operators, as in Appendix B). Ad libitum food resumes on the evening of day 28; the dummy caps are sealed and inspected after each session, swim speed is reported by arm, a rat that needs a rest day (core temperature $<34.5^\circ\text{C}$) has its probe and visible-platform test run on the same day at least 1 h apart, or is excluded and flagged; temperature is taken with subcutaneous transponders (IPTT) or infrared, not repeated rectal measurement; distal cues are large and high-contrast for albino rats at 10–20 lux.
35–36	Under anaesthesia an ICV dye injection (3 μl) is made where the cannula is patent (a blocked cannula is recorded; placement is judged from the cannula track, and resistance and backflow are recorded at every bolus), then decapitation (whole cage pairs, 8 + 7 pairs a day with two teams, an extra tissue person per team, ≤ 12 min per animal, balanced by arm and block; day 37 is a reserve day); a 2-mm coronal slab through the cannula track is cut fresh from the right hemisphere and photographed; the rest of the right hemisphere is snap-frozen and banked at -80°C ; the left hemisphere is cut into 1.5–2 mm slabs, fixed 48 h, cryoprotected in sucrose and banked as embedded blocks at -80°C (or in antifreeze at -20°C). Banked tissue is not analysed in this study.

Daily schedule and workload. The 30 rats are in five fixed blocks of six (two per arm). A rat's sequence is: behaviour in its pre-dose window (set-shift test ≤ 2.25 h; training ≈ 1 h; Hebb-Williams ≈ 0.5 h; water maze ≈ 0.7 h for the block) → 7,8-DHF/vehicle injection → ELE/control session 30 min later (60 min) → individual ration 2 h after the session ends. Block injections are at 2.5, 3.75, 5.0, 6.25 and 7.5 h after lights-off, and behavioural starts are staggered by 75 min per block, so each block has exactly 2.5 h from its behavioural start to its injection (a 2.25-h set-shift test leaves 15 min for transport and set-up; block 5 may start early when a test overruns); the last ELE session ends 9 h and the last ration falls 11 h after lights-off, inside the 12-h dark phase. Block b starts at lights-off + 75 (b–1) min, so about 4–6 testers are needed at once. Hebb-Williams and the water maze use dim white light (≈ 10 –20 lux, rooms only), a declared exception to the red-light rule (albino rats see little under red light). Six arenas are used (four ELE, two control), one operator per two arenas plus a fourth ELE helper for resets, so the working day spans about 13 h (lights-off minus 1 h to +12 h) covered by two

staggered shifts; the host states the clock times of lights-off and the 37 consecutive days (including weekends) on its roster, with a backup for the unblinded technician. Ordinary days need about 30 operator-hours (about 6 people, including four ELE staff); set-shift test days about 67 (about 12–15 people at the busiest hours, with ELE staff, the blinded injector and weigher, and testers kept separate; ration and weighing staff do not score). Day 0 is a Thursday so the peaks fall on Tuesday–Friday and Monday–Thursday; the weekend rota repeats the same schedule. In-life: arrival day –22 to euthanasia day 36 = 59 days (8.4 weeks). Phase 0a (reagents, tolerability, satellites) comes first; its calendar is in section 9. The fixed task orders (A: set-shifting → Hebb-Williams; B: Hebb-Williams → set-shifting; water maze last for both) are declared and the half is a fixed effect in the analysis; ELE and DHF after testing form a post-training consolidation window, declared. There is no fallback schedule: if the host cannot name the roster, the study does not start.

6. Endpoints and tissue

Primary analysis. The three task scores are standardised with the grand mean and the pooled within-arm SD estimated in-sample (no prior data exist), winsorised on residuals at ± 3 , signs aligned. They are analysed in one linear mixed model (task as a repeated factor; random intercept per animal; time block as a random intercept if it does not collapse to the boundary; half (task order) as a fixed effect; arm $\times$ task terms; baseline weight as the only covariate; common residual variance, with task-specific variances if the fit allows), using all animals with ≥ 1 task, with Kenward-Roger intervals (SAS PROC MIXED with DDFM=KR is the primary software; R packages are used only as a sensitivity check, because KR with heterogeneous variances is not available there). The gate statistic is the index contrast (mean of the three z-scores) $D+E - C$ in per-task-SD units, with $D+E - E$ and $E - C$ reported; set-shifting alone and the index without the water maze are reported separately. Sensitivity analyses: complete cases; tipping-point (δ -adjusted) analysis for informative dropout (the mixed model is valid under missing-at-random only). Wheel running is zero in arm C and post-restriction weight is post-randomisation, so neither is a covariate. Because ~ 18 –21 completers across three tasks may not support the full model, a simpler fallback primary is pre-registered: per-task contrasts ($D+E - C$, $D+E - E$, $E - C$) with t-based 95% intervals, combined as the unweighted mean of the three standardised contrasts. The index and this fallback always use the capped log set-shift score (time-censored rats at the cap); the censored (Cox) analysis is a separate set-shifting-alone readout, and the operating characteristics below no longer apply to the index if more than 20% of rats are censored. The inter-task correlation matrix is computed within arm from residuals and is descriptive. Analysis is run on coded labels by an analyst who does not hold the key (a third-party statistician if possible) until data lock.

Set-shifting score. Criterion 6 consecutive correct, maximum 60 trials per stage; the primary score is $\log(\text{trials to criterion at the extradimensional shift, capped at 60})$, with rats that do not reach it assigned the cap. If more than 20% of rats hit the cap at ED, the rank/censored (Cox) analysis becomes the primary set-shifting-alone analysis (the index and the OC table below assume the capped log score); the validity check ($ED > ID2$ cohort-wide) always uses a censored comparison. Conditioning on the post-randomisation ID2 score is a sensitivity analysis only. Water maze: mean proximity to the platform in the 24-h probe [21] (start-position correction stated by the host); quadrant time also reported.

Block	Readouts	n
Behaviour	Three tasks; scored live and verified from coded video.	all completers
Banked tissue (every main-cohort animal)	Left-hemisphere slabs fixed and banked (embedded blocks at -80°C or antifreeze at -20°C); right hemisphere frozen. No analysis in this study; reserved for Olig2/MBP histology and synaptic markers in the larger study (no EdU label is given in this study; immersion fixation gives lower and depth-dependent quality than perfusion — declared).	30
Engagement (satellites, snap-frozen)	pERK/total ERK in periventricular tissue (gate anchor) and hippocampus (descriptive); ice-cold dissection (<2 min), phosphatase inhibitors, FGF2-stimulated positive-control lysate and pre-specified assay CV on every blot; the same killing and dissection method in every group, which makes any stress- or anaesthesia-related change in pERK common to all groups (decapitation without anaesthesia needs CEUA justification under the CONCEA euthanasia guideline; published comparisons find isoflurane and unanaesthetised decapitation similar, and both can raise stress-related ERK signalling); one gel carries all 12 samples plus the bridging control so fold-changes are compared on one membrane; pERK and total ERK on the same membrane (strip and re-probe, validated) or a parallel gel.	4 + 4 + 4

7. Statistics and pre-registration

Register on OSF before surgery (sponsor does this); ARRIVE 2.0 [14]; flow diagram of enrolled, excluded and completed animals. Estimation only: unadjusted contrasts with Kenward-Roger 95% intervals as primary; an animal-level (cluster) bootstrap that re-runs standardisation, winsorising and the model in every replicate is reported as sensitivity; no significance claims are made. The task order halves are balanced by arm, so the main contrast is protected, but arm $\times$ half is not usefully estimable. Sizing of the next study: use the upper 80% confidence limit of the pilot's raw-unit residual SD per task [20] and the full factorial/interaction design.

8. Pre-registered decision rules and operating characteristics

Phase	Pass condition
0a – reagents	FGF17 bioassay active on the pack; mouse-rat alignment documented; DHF vehicle stable and tolerability screen passed (switch rule applied); potency recheck passed before the main cohort.
0a – FGF17 engagement (gate; signalling at the ventricular interface – necessary, not sufficient; the hippocampal punch is the descriptive parenchymal-penetration readout)	Periventricular pERK/total ERK, ratio of group means, FGF17 $\geq 1.5 \times$ time-matched vehicle AND FGF17 $\geq 1.25 \times$ heat-inactivated FGF17, at 30 min (n = 4 vehicle, 4 FGF17, 4 heat-inactivated; point estimates; acceptance CV pre-specified; positive-control lysate on every blot; the threshold applies to the diluted punch tissue actually assayed). A borderline result (FGF17/vehicle ratio 1.3 to <1.5) triggers one re-test (fresh 4 + 4 + 4 rats at 30 min, same pass rule; the decision rests on the fresh sample alone). Operating characteristics (200,000 simulations; lognormal assay noise at CV 30% / 50%; effect present; heat-inactivated group at vehicle level): false-pass with the re-test $\approx 1.6\%$ / 7.8% if there is no effect; pass $\approx 26\%$ / 30% at a true 1.3-fold change; $\approx 57\%$ / 49% at 1.5-fold; $\approx 97\%$ / 83% at 2-fold. A true 1.5-fold engagement therefore passes only about half the time: the gate detects large engagement and can miss a real but modest one. These figures assume the heat-inactivated group sits at vehicle level and assay CV $\leq 50\%$: if boiling leaves partial activity (heat-inactivated at $1.5\times$ when the true effect is $2\times$) the pass rate at 2-fold falls to $\approx 64\%$ / 56% , and at assay CV 70% the pass rate at 1.5-fold falls to $\approx 45\%$ with a false-pass of $\approx 13\%$ (round-1 re-simulation by a reviewer). The heat-inactivated ratio is therefore the limiting criterion, and the boiled aliquot is bioassayed. The single 30-min time point is an assumption anchored only by the FGF2 control curve and an FGF1 ICV precedent in hypothalamus (pERK detectable from ≈ 20 min and sustained for hours; a different ligand and region, so an extrapolation to FGF17 at the periventricular wall); if the true time course is different the gate can miss it. FGF17, an FGF8-subfamily ligand, is expected to signal through c-isoform FGFRs (FGFR3c/FGFR4, per Zhang 2006 [23]; FGF17-specific profile to be verified); the cited oligodendrocyte-lineage FGFR study tested other FGFs, not FGF17 [16]. If the gate is not met, the main cohort does not start; test a higher dose or continuous infusion in a small follow-up. An FGFR-inhibitor arm is deferred to the follow-up experiment.
0b – feasibility	$\geq 50\%$ of the 30 enrolled rats (≥ 15) complete all three tasks; cannula placement $\geq 75\%$ correct. (Binomial $P(\geq 15/30)$: 0.77 at 55%, 0.90 at 60%, 0.97 at 65%, 0.99 at 70% true completion; a single wave, so no wave clustering.)
0b – direction	Index contrast $D+E - C \geq +0.5$ per-task SD (in-sample pooled SD; a small circularity that is declared) and $D+E \geq E$ in point estimate. The gate applies to the point estimate; the interval is reported alongside and is not itself a gate. A reported, non-gating transfer flag records whether the set-shifting contrast $D+E - C$ and $D+E - E$ are both ≥ 0 (adding it as a requirement would lower the target-scenario go rate from about 72% to 63%).
Provisional go	Phase 0a passed AND 0b feasibility AND 0b direction. Joint probability, illustratively assuming independence and 90% feasibility: $\approx 63\%$ if FGF17 truly engages at 2-fold and the target effect holds, $\approx 37\%$ if engagement is 1.5-fold; $\approx 10\%$ if FGF17 engages but behaviour is null; $\approx 0.2\%$ under a true global null (CV 30%; larger at CV 50%). Differential cap loss can select fitter $D+E$ completers and bias the direction upward; the arm-stratified tipping-point analysis addresses this.

Operating characteristics of the direction rule (simulated; 6 completers per arm | 7 completers per arm; three tasks; index contrast; r = inter-task correlation; percentage that would be a “go” on direction alone):

True situation	$r=0$	$r=0.3$	$r=0.6$
No effect	7% 5%	11% 9%	13% 12%
Modest (E 0.3, D+E 0.5)	45% 45%	43% 43%	41% 42%
Target (E 0.3, D+E 0.8)	79% 81%	72% 74%	67% 69%
Large (E 0.5, D+E 1.2)	97% 98%	92% 94%	87% 89%
Enrichment alone (E 0.6, D+E 0.6)	39% 40%	39% 38%	38% 38%

The rule is lenient (about 10% false-go) and cannot tell the drug package from enrichment: enrichment alone gives a “go” in about 39% of runs. The table assumes the same effect on all three tasks: if the effect is on set-shifting only (the clean transfer readout) the go rate is only about 25% at the target size, and if it is on the two spatial tasks only about 48%, so the index can be driven by the spatial tasks and miss a transfer-specific effect ($r = 0.3$, 6 completers). It only screens for a promising direction. A “go” justifies the larger study only; a “no-go” identifies the failure mode (delivery, engagement, ceiling, attrition) and is not evidence that the approach cannot work.

9. Brazil logistics and budget

Item	What the review found
Ethics	Animal use is approved by the host institution's CEUA (Comissão de Ética no Uso de Animais) under Lei 11.794/2008 (Lei Arouca), Decreto 6.899/2009 and the CONCEA normative resolutions. The institution, its CEUA and its animal facilities are registered in CIUCA, and the institution must hold a valid CIAEP (CONCEA accreditation, RN 50/2021, valid 5 years); check both on the CONCEA list before choosing a host, because a lapsed accreditation bars animal work. Invasiveness is graded under CONCEA RN 55/2022 Annex II (the cannulation surgery is expected to be Moderado, G2; the CEUA may grade the cumulative burden higher, G3 or G4, "grave"; CEUA forms that still use G11-G14 would call it G13; the host confirms the current labels). Every person on the protocol needs documented training (CONCEA RN 49/2021, about 30 h suggested by CONCEA guidance, covering ethics, welfare and procedure-specific training; RN 39/2018 appears to be revoked, to be confirmed). Review time differs by institution (1-6 months; a reviewer assumption); ask for an interim review. The principal investigator must be host staff. Animals are reserved only after approval; practice and contingency rats are in the approved protocol; the researcher reports to the CEUA on the institution's form, and the institution and CEUA report annually to CONCEA through CIUCA, and carcass and biological-waste disposal follow ANVISA RDC 222/2018 and CONAMA 358/2005.
Imports	FGF17 comes from one vendor and the antibodies and cannula sets from others, so plan two import processes (or a local distributor, whose mark-up can be large): about 4-12 weeks each including licensing, customs and cold chain (reviewer assumption), customs-broker (despachante) fees, dry-ice courier. Federal relief: Lei 8.010/90 (II, IPI) and, until 31 Dec 2026, PIS/COFINS-import relief (Lei 10.865/2004; CNPq Portaria 2.248/2025), for imports by CNPq-accredited institutions or researchers who are the importer of record. LC 224/2025, already in force, cuts federal tax benefits by 10% (an exemption becomes about 10% of the standard rate unless excluded; II and IPI/PIS/COFINS steps apply from 2026), so even exempt imports may pay roughly 2-3% of customs value if research imports are not excluded (unconfirmed). From 1 Jan 2027 PIS/COFINS-import are replaced by the new consumption taxes (CBS, LC 214/2025) and IPI is largely zeroed (sources differ; confirm in the consolidated text); whether CBS/IBS relief applies to CNPq research imports must be confirmed with the host's import office or Receita Federal. The licence is registered in Siscomex and needs CNPq approval (a figure of up to 60 days was quoted by one source but not verified); some foundations now require the commercial invoice, bill of lading and packing list for the CNPq procedure, so confirm whether the licence must precede payment. ICMS relief (Convênio ICMS 93/98, with CNPq-accredited researchers added by Convênio 57/2005; each state adopts it by its own norm) applies only if the import is also exempt from II/IPI and, for laboratory articles, requires an attestation that no similar product is made in Brazil (issued by a federal body or a national industry entity, or by a state body for parts and laboratory articles; valid for at most 6 months) plus the state's own procedure (some states require annual credentialing); whether reagents and antibodies count as laboratory articles is to be confirmed, and losing the CNPq exemption also loses the ICMS relief. Reagents under sanitary control follow ANVISA RDC 172/2017 (automatic approval for CNPq-accredited importers, except human biological samples and substances controlled under Portaria SVS/MS 344/98; amended by RDC 613/2022 and RDC 988/2025). Importa Fácil Ciência (Correios; postal origin only, ≤30 kg; the current value cap was not confirmed) is probably unsuitable for dry-ice cold chain. Quota and calendar risk: the annual CNPq quota is regularly used up (2025: USD 229M, frozen mid-year; 2026: USD 254M, exhausted in June and raised to ≈USD 354M in July; the 2027 quota is normally set by a Portaria MF in November-December). File the licence early in the calendar year, and keep a fallback: the institution's direct import outside the quota where its status allows, or a local distributor at the tax-paid price. If the exemption is refused, the imported lines (≈3.6k landed) cost more, a risk scenario estimated at +1.0-1.5k (unverified) shown separately below. A pending bill (PL 2.374/2019) would remove the quota cap on federal taxes; it is a watch item. Purchases with foundation-managed funds follow the foundation's procurement rules (quotes, a sole-source justification for single-vendor FGF17, a handling fee per import), which adds weeks to the order calendar. The base budget assumes landed cost ≈1.3-1.5× list. Rates here come from search snippets and reviewer knowledge, not verified.
Controlled drugs	Ketamine (list B1 since 2022), buprenorphine and other opioids, and tramadol (list A2) are controlled (Portaria SVS/MS 344/98; xylazine is controlled on MAPA's veterinary list). The host states its route: (i) MAPA-registered veterinary products bought and prescribed by a CRMV-registered veterinarian through SIPEAGRO (MAPA IN 35/2017), or (ii) human-use products under an ANVISA special authorisation (AEP, RDC 367/2020 as amended), which covers strictly academic use only (a paid service contract may instead need a full AE under RDC 16/2014), does not authorise import (a separate import authorisation is needed) and requires movement reports (BSPO). Both routes need a locked store and a stock register; the host checks the current Portaria 344 annexes and MAPA IN 35/2017 as updated (Portaria MAPA 837/2025). This protocol plans isoflurane for anaesthesia and meloxicam, dipyrone and local infiltration for analgesia, and uses controlled drugs only through the host's authorised route.
Equipment access (precondition)	The budget assumes written access to: stereotaxic frames (two), a microinjection pump, a water maze with tracking software, a cryostat or vibratome only if banked tissue is later processed, a western blot system and image-analysis software. Multi-user facilities in Brazil often charge hourly or require a contrapartida; those fees are NOT in any scenario here and must be quoted. The ELE arenas, Hebb-Williams mazes and digging boxes are priced as fabricated in the base case; the optimistic case assumes the host owns them.

Item	What the review found
Personnel, services, overhead	Not in the budget. About 2,420 operator-hours (Appendix B), a coordinator (about 4 months), a surgeon, a veterinary lead and an unblinded technician. Operator cost depends on how people are engaged: graduate students on CAPES/CNPq stipends may take extra paid work only within the accumulation rules and with advisor approval; under a partnership agreement (Lei 10.973/2004 art. 9) servants and students may receive research bolsas, which are not employment, whereas under a service contract (art. 8) students cannot receive bolsas, host servants may receive only an adicional variável paid from the contract's revenue, and other labour is CLT or autonomous with payroll charges; weekend premiums (CLT Sunday work carries a premium and compensatory rest) are part of the unquoted personnel cost. Unverified rates: bolsa equivalents roughly R\$13–20 per hour (master's/doctoral stipends) and loaded CLT technicians R\$35–50 per hour (assuming about R\$5.1–5.2 per USD in October 2026), i.e. operators about USD 9–22k for about 2,420 operator-hours. Adding a coordinator (USD 4–6k), other staff and equipment fees (USD 1.5–4k) and institutional overhead (institution-specific; reviewer examples 5–20%, with caps of 15–18% found at some foundations), the plausible all-in total is USD 25–55k (an unverified estimate). Student stipend rules, dedication limits, night and weekend access to the animal facility and thesis/authorship expectations must be agreed with the host.
Timeline	Contracting and the work plan (2–6 months, not counted); CEUA 1–6 months; with FGF17 ordered at CEUA approval, Phase 0a runs about 24 weeks before the cohort arrives (calendar below), then 8.4 weeks in-life and about 1 month of analysis: about 9–14 months from CEUA submission to core results (about 7 months if FGF17 is ordered and bioassayed before approval, which needs the host to accept the risk). A triggered re-test adds 5–6 weeks. Institutional risks (strikes, closures) are a host-selection criterion.

Phase 0a calendar (planning offsets in weeks before the main cohort arrives on day –22; the host confirms): CEUA approval and FGF17 order (–24); pack arrives, bioassay of the pack and the boiled aliquot (–14 to –12); tolerability rats start dosing (–14) and finish their dosing day 35 (–9); satellite and practice surgeries (–12 to –11) with dosing and tissue at –10 to –9; westerns and gate readout (–8 to –6); written decision (–5, with the placement score of all 14 satellite and practice rats); main-cohort rats ordered (–4, after the gate and the vehicle decision; age-matched males need provisional reservation at CEUA approval); the second FGF17 lot is reconstituted about 5 weeks before day 0 so that no aliquot is older than about 4 months at the last bolus. Satellite and pilot surgery never shares a day with main-cohort surgery.

Budget (USD; list prices from search plus reviewer knowledge; to be replaced by quotes):

Line (core = Phase 0a + 0b)	Base	Notes
Rats: 48 (30 main, 12 satellites, 6 practice/tolerability) at external-rate prices, with transport	1.0k	Public lists found: USP PUSP-RP Wistar Hannover R\$62.50 external (2022; ICB/USP R\$60); UFMG Biotério Central 2026 Wistar Hannover external R\$57.60 (≤5 wk) to R\$94.80 (8 wk), internal R\$20–66 (rat column to be confirmed). Internal rates apply only to internal users.
Housing/care (≈2,250 rat-days — not operator-hours — ≈ 1,100 divider-cage pair cage-days, reversed-cycle room)	1.0k	Brazilian per-diem not found (ask the facility); an Indian price list gave USD 0.47–0.94 per cage-day (used only as an order of magnitude); ICB-II/USP internal about R\$1 per cage-day; optimistic 0.55k, pessimistic 1.4k; doubles if pair-housing is refused
FGF17, one 50 µg pack, landed	0.65k	Cohesion ≈ USD 480/50 µg via an aggregator; R&D; 25 µg mouse vial price not found; PeproTech lists human only; a 25 µg contingency vial 0.45k in pessimistic
7,8-DHF (purchase ≈2 g for pack size and margin; need ≈1 g) and vehicle components, landed	0.25k	TCI ≈ USD 62/g
Cannulas, guides, dummies, injectors, screws, cement (44 rats), microinjection tubing	0.8k	Commercial sets ≈ USD 20–30 each landed (1.1k); home-made guides from steel tubing ≈ 0.4k; divider cages and split hoppers are in small consumables
Anaesthesia, analgesia, surgical and terminal consumables (44 cannulations, 48 terminal procedures), including controlled-drug handling	0.5k	
ELE: 6 arenas (4 ELE, 2 control) ≥1 m², solid wheels, tunnels, objects, six-level puzzle ladder	1.8k	Fabricated locally ≈ USD 150–300 per arena; get two quotes; 0.4k if the host owns the arenas
Behaviour: 2 Hebb-Williams mazes, ≥10 digging boxes in two rooms, set-shift pots, media and odours (≥10 exemplars per dimension)	1.0k	0.5k if the host owns the mazes and boxes
ERK block, landed: pERK, total ERK, HRP secondaries, loading control, FGF2 positive control	1.55k	list ≈ 1.15k × 1.35
Western reagents (membranes, ECL, inhibitors)	0.35k	
Two import processes with customs-broker and courier fees	0.7k	Brazilian broker and storage fees not quoted

Line (core = Phase 0a + 0b)		Base	Notes
Small consumables: Hamilton syringes, aCSF, BSA, heparin, low-bind tubes, opaque sleeves, 3T3 cells and culture, food/reward pellets and bedding if billed, cameras (infrared) and storage, red/white lights, subcutaneous temperature transponders, perforated cage dividers, tempera paint, dye, fixative, sucrose, cryovials, waste, dry ice		1.0k	Reviewer estimate 0.8–1.5k
CEUA fee and miscellaneous		0.1k	
Contingency rats (12 registered re-test rats: purchase, housing, surgery)		0.5k	Spent only if the re-test is triggered
Core base total, materials only		11.2k	excludes personnel, overhead, equipment fees, FX reserve
Scenario	Core	Assumptions (itemised)	
Optimistic	≈ 8.0k	host owns arenas, mazes and boxes (–1.9k); host animal facility at internal rates (rats –0.35k, housing –0.45k); antibodies at list (–0.15k); home-made cannula guides (–0.4k)	
Base	≈ 11.2k	as above	
Pessimistic	≈ 13.3k	15% overrun plus a contingency 25 µg FGF17 vial; equipment fees, personnel and overhead still excluded	
Pessimistic plus refused CNPq exemption	≈ 14.3–14.8k	imported lines taxed (+1.0–1.5k, unverified); also likely months of delay	

Ceiling (researcher-approved): USD 12k for materials. The base (11.2k) and optimistic (8.0k) cases are inside it only before a 10–15% currency reserve (≈1.1–1.7k, not in any line; October 2026 rate assumed R\$5.1–5.2 per USD), which would push the base to about 12.3–12.9k; the pessimistic cases (13.3k and 14.3–14.8k) exceed it. The reserve, personnel and overhead sit outside the materials ceiling by this protocol's definition; the sponsor should decide whether to raise the ceiling to about 13k. Phases (line by line): 0a ≈ 5.4k = rats 0.4 + housing 0.35 + FGF17 0.65 + DHF 0.1 + cannulas 0.3 + surgical 0.2 + ERK 1.55 + western 0.35 + imports 0.5 + small consumables 0.4 + CEUA 0.1 + contingency rats 0.5, with a stop rule at that spend; 0b ≈ 5.8k = rats 0.6 + housing 0.65 + DHF 0.15 + cannulas 0.5 + surgical 0.3 + ELE 1.8 + behaviour 1.0 + imports 0.2 + small consumables 0.6. First cuts if prices run high: use host-owned apparatus; home-made cannula guides; do not register the contingency rats (a smaller arm size would need the block design redone and is not recommended). The western block must not be cut (it carries the only gate). Must not be cut: placement check, FGF17 bioassay and alignment, blinding/randomisation, DHF tolerability screen, visible-platform control, banking of tissue.

10. Lab handover and remote oversight

The researcher will not necessarily be on site; the host laboratory runs the experiment. This PDF is the working protocol; the following must exist before animals arrive. SOPs, CEUA forms and scoresheets are kept bilingual (Portuguese for staff; English for the sponsor).

Item	Responsibility / requirement
Legal form (decide first)	Either (a) an academic partnership with the host institution (an acordo de parceria para PD&I; under Lei 10.973/2004 art. 9, regulated by Decreto 9.283/2018; research bolsas for students are possible under this route), usually with funds received and managed by its research-support foundation (fundação de apoio, Lei 8.958/94, Decreto 7.423/2010) as intervening party; the foundation must be currently accredited by MEC and MCTI; the overhead is set by the institution; IP and data ownership are written into the instrument (shared per the agreement) and the host's technology-transfer office (NIT) is usually involved; or (b) a service contract (prestação de serviços, Lei 10.973 art. 8) with an invoice (nota fiscal), ISS and taxes agreed; results then usually belong to the contracting party, which the contract must state. Route (b) probably loses three assumptions of the base budget at once (the Lei 8.010 import exemption, which is tied to research by the accredited importer; the AEP, which covers strictly academic use; and student bolsas), so route (a) is the only route consistent with the 11.2k base; route (b) must be re-costed with full taxes, an AE and adicional-variável or CLT pay. For state universities (for example São Paulo) the state innovation law and the university's foundation rules apply instead of the federal decrees; the host states which regime applies. Which route pays which people is written into the instrument. The sponsor is an individual: the protocol must name the legal entity through which the money enters (a sponsor company, a foreign institution, or a donation to the foundation), and the host's legal office (procuradoria) must confirm it can contract with that entity before this step. The sponsor's residence (in Brazil or abroad) decides currency exchange, IOF (residual rate 0.38% on inbound FX under Decreto 12.466/2025), the foreign-exchange contract nature code, withholding and, for a donation, possible state ITCMD (public ICTs and non-profit scientific institutions may be immune; the foundation confirms) and must be stated; whether goods imported duty-free under Lei 8.010 may be used in a paid service contract must be confirmed with CNPq or the import office. Foreign funds arrive by international transfer with the usual exchange documentation; no restriction on foreign funds is known to us for this kind of contract, but confirm in writing with the foundation. The choice determines payment mechanics and is a precondition for CEUA submission.

Item	Responsibility / requirement
Host selection and due diligence	Criteria: institution with a valid CONCEA CIAEP, a functioning CEUA and a registered animal facility; a named route for controlled drugs (MAPA/SIPEAGRO veterinary purchase or an ANVISA AEP) with a locked store, a stock register and a CRMV-registered veterinary lead; a CNPq-accredited importer of record and a plan for the quota calendar (file the licence early in the year, with the fallback in section 9); a current Federal Police chemical-control licence (CLF) covering assay reagents; the rooms listed below; a dedicated reversed-cycle room, six arenas, a water maze, two Hebb-Williams mazes and a separate euthanasia room; written record of ICV cannulation, set-shifting and water-maze work; access to all listed equipment; references from two previous external collaborations; a named roster of operators (with RN 49/2021 training records; about 12–15 people at the busiest hours, about 6 on ordinary days) and a facility-hours map for nights and weekends; stable operation (no recent strike disruption); a foundation with a current MEC/MCTI accreditation and a stated foreign-exchange procedure. Rooms and equipment: a dedicated reversed-cycle colony room; two ventilated set-shift rooms; a Hebb-Williams room; the water-maze room; six ELE/control arenas; light-tight or covered transport between rooms; infrared cameras and illuminators; a separate euthanasia room; the clock times of lights-off stated. Trade-off: a CRO offers fee-for-service and quality systems but rarely these assays; an academic lab offers expertise but expects authorship and slower timelines.
Payment tied to gates	Public hosts and foundations generally spend only deposited funds, so the first tranche must pre-fund Phase 0a: (1) at signature (CEUA fee, practice and satellite rats, the FGF17 pack and ERK antibodies, surgery, personnel for 0a); (2) after the written 0a gate decision (including the 14 placement scores), the main cohort; (3) at data lock, a small holdback. A failed 0a stops spending at its tranche; staff commitments on monthly stipends are stated for that case. The agreement states who bears currency risk: budget in BRL, convert per tranche, and hold a 10–15% FX reserve (or agree USD tranches with a renegotiation band); bank spread and IOF are included.
Governance and welfare oversight	Protocol amendments need the host PI, the sponsor and the CEUA; the veterinary lead can stop the study; weekly remote welfare report (weights, scores, cap losses, adverse events); escalation: sponsor within 48 h, CEUA coordinator and unit head within 7 days; fixed weekly call slot.
Quality standard	Non-GLP; the host works to GLP-like good practice (contemporaneous records, calibrated equipment, training records, retained raw data).
Roles and accountability	Host PI (accountable for CEUA compliance and animal welfare; also the CEUA PI); study coordinator (one named person); about six operators on ordinary days and about twelve to fifteen at the busiest hours, in two staggered shifts as in section 5 (ELE staff, blinded injector, testers and preparer kept separate), two surgeons and a weekend rota; an unblinded technician outside the testing team who prepares coded syringes; sponsor (design, randomisation, drug-code key, registration, independent analysis).
Agreement terms	Funding, overhead and payment schedule; who pays for overruns and for a failed Phase 0a; IP ownership; data and tissue ownership; insurance and liability; stop rules; publication and authorship (ICMJE criteria); data sharing.
Host-competence evidence	Before signature: the lab's written records of ICV cannulation, set-shifting and water-maze work, its western blot CV from previous work and a verified equipment list. Before the main cohort: $\geq 80\%$ correct cannula placement (≥ 12 of 14) across the 12 satellite and 2 practice rats, scored from the fresh coronal slab through the cannula track (if it fails, the sponsor funds a short training and re-test or reassigns the host, and no main-cohort money is spent until it is met). Sponsor decides go/no-go on this evidence.
SOP pack (host completes with photos; sponsor reviews)	Surgery and cannula care; ICV bolus injection; vehicle preparation and dosing; ELE and control sessions; set-shifting; Hebb-Williams; water maze incl. temperature/hypothermia rules; food restriction and weighing; euthanasia, tissue banking; satellite dissection and western protocol; per-day time-budget table; welfare scoresheet; data-recording SOP; Appendix A parameters signed off. The CEUA application is drafted jointly: the sponsor supplies a model narrative (severity, endpoints, justification), the host adapts it.
Blinding and randomisation	Sponsor generates the allocation and the coded vial/arm labels (an unblinded host technician outside the testing team prints and fills them), and keeps the key; host testing staff handle coded IDs only; unblinding after data lock.
Data integrity and monitoring	Bound or electronic lab notebooks with audit trail; raw files hashed on upload; protocol deviations classed minor/major with major ones reported within 48 h and a sponsor decision; scheduled live-video audits and source-data verification of $\approx 10\%$ of records; weekly summary (enrolment, weights, exclusions, adverse events, cap losses); raw data and images shared with the sponsor for independent blinded analysis.
Tissue transfer	Banked tissue stays at the host unless a material transfer agreement and the necessary export permits are in place (export permit requirements to be confirmed).
Decisions	After 0a and 0b the sponsor issues a written decision (provisional go / stop / redesign) before further spending.

Appendix A. Task parameters (defaults; host to confirm and sign off before animals arrive)

Task	Parameters
Set-shifting (digging; Birrell & Brown 2000; Tait et al. 2018 [11,19])	Two ceramic pots per trial; stimulus dimensions: digging medium and odour (neither is used in the ELE); 3 days digging habituation to a baited pot; example exemplar sets (host to fix ≥ 10 each before animals arrive, because simple, compound, ID1, ID2 and ED each need new pairs and the training pairs are separate; drop near-duplicate media such as sawdust and wood chips): odours — cinnamon, nutmeg, clove, cumin, thyme, vanilla, coriander, ginger, paprika, anise; media — shredded paper, sand, plastic beads, cotton, fine gravel, foam pieces, felt squares, wood shavings, paper pellets, rubber crumb (pairs are matched for salience in the practice rats; dimension order is counterbalanced 3:2 across the 5 rats per arm in each half and recorded); stages: simple discrimination, compound discrimination, first intradimensional shift (ID1), second intradimensional shift (ID2, to build the attentional set), extradimensional shift (reversal stages omitted to limit session length — declared); validity check: ED trials > ID2 trials cohort-wide; dimension order and exemplar sets counterbalanced across rats and arms; odour applied to the medium and bait buried in both pots (masked), with a fixed discovery-trial rule; criterion 6 consecutive correct; maximum 60 trials per stage; 3-min trial limit; test cap 2.25 h per rat per day; error on first incorrect dig; bait $\approx \frac{1}{2}$ cereal piece; pot positions pseudo-random; training cap 5 days.
Hebb-Williams (Rabinovitch & Rosvold 1951 [22])	Closed-field maze with barrier layouts; 2 days adaptation with food reward including the standard practice problems; 4 test problems chosen from the standard 12 by host and sponsor before animals arrive (fixed order, same layouts for all rats; investigator-chosen subset, not validated); 6 trials per problem (3-min cutoff), one problem per day (4 days), two identical mazes in parallel; error zones defined in advance; split-half reliability reported; dim white light (≈ 10 –20 lux) in the maze room, as for the water maze.
Morris water maze	Pool ≈ 150 –180 cm, 25 ± 1 °C, hidden 10-cm platform 1.5–2 cm below the surface in opaque water (non-toxic black tempera paint, for contrast with albino rats), one pool, ≥ 4 distal cues, dim white light (≈ 10 –20 lux, measured; spectrum recorded) in the pool room only — a declared exception to the dark-phase red-light rule; rats travel in covered cages and the colony room stays dark; 4 trials/day, 60 s maximum, four quasi-random start points, ≥ 5 min between trials in a warm cage (32 °C); core temperature before and after sessions (rest day if <34.5 °C); 4 acquisition days; 60-s probe 24 h after the last acquisition day; visible-platform test (4 trials) after the probe; scores: mean proximity (Gallagher), target-quadrant time, swim speed, floating, thigmotaxis.
ICV cannula (to be confirmed in the practice group)	Right lateral ventricle, rat aged 8–10 weeks (≈ 250 –300 g): from bregma AP -0.9 mm, ML $+1.5$ mm, DV such that the injector tip lies at -4.0 to -4.5 mm (the host states whether DV is measured from bregma or the skull surface; guide ≈ -3.0 to -3.5 mm with the injector protruding ≈ 1 mm), scaled by the bregma-lambda distance and confirmed by dye in the practice group; fixed with screws and dental cement under a protected recessed cap; dummy cannula between injections.
FGF17 bioassay (0a reagent gate)	Serum-starved FGF-responsive cells (the supplier's BALB/3T3 line, or NIH-3T3; primary rat OPCs if available) stimulated for 10–15 min with 0–100 ng/ml of the supplied FGF17 in the presence of heparin 10 $\mu\text{g/ml}$ (an EC50 comparison is only valid with matching heparin); pERK/total ERK by western; FGF2 as positive control; pass = a clear dose-response with ≥ 3 -fold maximal pERK over unstimulated cells; the supplier's stated ED50 is a proliferation value, so on first purchase the pERK dose-response is judged against FGF2 and the ≥ 3 -fold criterion (an earlier-lot comparison is only possible for later lots). A 3T3 pass confirms potency through FGFR1c/2c, not OPC-relevant FGFR3/4 signalling. The boiled (heat-inactivated) aliquot is run in the same assay and must show <10% residual pERK activity and no visible aggregation.
Antibodies and kits (host to confirm; unverified examples from memory)	pERK1/2 Thr202/Tyr204 (e.g. CST #4370), total ERK1/2 (e.g. CST #4695); the host lists final catalogue numbers, rat reactivity and dilutions in the SOP pack.
ELE / control session	Dark phase under red light; solo session of 60 min (no partner); 6 arenas (4 ELE, 2 control), arena identity rotated by day within type; arms balanced within every time block (two rats per arm per block); log object list, puzzle level, rewards, engagement minutes, wheel revolutions.

Appendix B. Operator time budget (to be confirmed by the host)

Period	Daily work	Operator-hours/day
Days –15 to –13	Cannula surgery, ≤10 rats/day on two frames (two surgeons + two assistants, ≈8 h each)	≈ 32
Days –12 to –1	Handling and weighing ≈ 8 h/day; arena habituation from day –7 (30 rats in an empty arena) adds ≈ 12 h/day for 7 days	≈ 8 / 20
Days 0–34, every day incl. weekends (routine)	ELE/control: 30 solo sessions in 6 arenas over five blocks (≈6.5 h of sessions; one operator per two arenas ≈ 20 h, plus a fourth ELE helper for resets ≈ 4 h); injections ≈ 1.5 h; weighing, individual rations and cage checks ≈ 1.5 h; cleaning and object preparation ≈ 2 h; logs and video ≈ 1 h	≈ 30
Days 0, 2, 4, 6 (extra)	ICV boluses (30 rats × ≈10 min × 2 operators)	+ 10
Days 5–10	Food restriction ramp, extra weighing	+ 2
Days 11–16	Half A set-shift habituation/training (15 rats × ≈1 h); half B Hebb-Williams (15 rats × ≈0.5 h)	+ 22 (total ≈ 52)
Days 17–18	Half A training 15 h; half B set-shift habituation 15 h	+ 30 (total ≈ 60)
Days 19–22	Half A set-shift tests: up to 10 rats testing × ≤2.25 h ≈ 22 h (days 20–21 are the busiest; at most about 4 testing at one moment); half B habituation (day 19) or training (days 20–22) ≈ 15 h	+ 37 (total ≈ 67)
Days 23–24	Half A Hebb-Williams ≈ 8 h; half B set-shift training ≈ 15 h	+ 23 (total ≈ 53)
Days 25–28	Half A Hebb-Williams ≈ 8 h; half B set-shift tests ≈ 22 h	+ 30 (total ≈ 60)
Days 29–34 (water maze)	30 rats × 4 trials, interleaved by block, two core temperatures ≈ 9 h with two operators	+ 18 (total ≈ 48)
Days 35–36 (euthanasia)	whole cage pairs (8 + 7 a day) with two teams, an extra tissue person per team, ≤12 min per animal; no routine (ELE and dosing end on day 34)	≈ 12
Weekends	The same task schedule repeats on weekends (day 0 on a Thursday)	as the weekday row
Phase 0a (before the cohort)	14 surgeries ≈ 35 h; tolerability dosing and palpation (35 days) ≈ 35 h; satellite dosing and dissection ≈ 15 h; bioassay, potency and westerns ≈ 80 h; training and set-up ≈ 85 h	≈ 250 in total
Video verification	Set-shift and Hebb-Williams sessions verified from video and 20% re-scored	≈ 150 in total

Total ≈ 2,420 operator-hours: the main cohort rows sum to about 2,020 (routine 35 × 30 = 1,050; extras 40 + 12 + 132 + 60 + 148 + 46 + 120 + 108 = 666; surgery 96; handling and habituation 180; euthanasia 24), plus Phase 0a ≈ 250 and video ≈ 150. The peak is ≈ 67 operator-hours a day on set-shift test days (days 19–22). Because dosing is staggered over 5 hours and behaviour starts are staggered by block, about 4–6 testers are needed at once; with ELE staff (4), the blinded injector and weigher (2) and the other half's operators (2–3), the busiest hours need about 12–15 people in two staggered shifts, and ordinary days about 6. Weekend and CLT premiums are part of the unquoted personnel cost. Personnel cost is not in the budget.

11. Validation record

The mini design inherits its task methods, assay logic and many safeguards from a larger design reviewed in the rounds below (the early rounds were run in the India form). Reviewer panels were 5 Opus + 2 Sonnet agents per round unless stated. Reviewer statements are model judgements, not independent human review.

Round	Reviewers	Verdicts	Main issues → fix
Earlier A, B, translational	3 Opus, 3 Opus + 1 Haiku, 1 Opus	All approve with changes	Pump ends before testing, pump cost, middle-aged rats, transfer overlap, social confound, composite not a g factor, overclaiming, neurotrophin history, FGF safety → boluses, young adults, staged gates, hedged roadmap.
India-form rounds 1–8	5 Opus + 2 Sonnet each	All approve with changes every round	Gate calibration, slab fixation, vehicle choice, sequential waves, fixed slots, set-shift scoring, staffing arithmetic, frozen-SD rule, Cox primary, time-matched vehicle, heat-inactivated control, tolerability, budget chain, many wording fixes (history of the larger design).
Brazil full-size round B1	5 Opus + 2 Sonnet	All approve with changes; budget and executability: major revision	Housing and rats underpriced; all-in cost unquantified; wrong legal instruments; CNPq quota exhausted in 2026; controlled-drug licences; dark-phase arithmetic on test days; weekend contradiction; staffing below concurrency; ill-defined gate and re-test → this mini redesign (smaller, one wave, solo ELE sessions, parallel halves, gate and OC re-simulated, budget re-itemised).

Round	Reviewers	Verdicts	Main issues → fix
Mini round M1	5 Opus + 2 Sonnet	All approve with changes; behaviour: not executable as written (blocking, fixable on paper); budget: all-in cost understated	Daily block model could not carry the 25–30 pre-dose sessions → dosing ladder staggered 2.5–7.5 h after lights-off with fixed blocks and divider cages; day 0 on a Thursday; ≥10 set-shift exemplars and stage-failure/time-cap rules; budget re-itemised (base 7.3k → 11.2k: arenas and mazes, cannulas, two imports, small consumables); all-in 25–55k; tramadol and controlled-drug route, RN 55/2022 grading, 2027 CBS tax regime, art. 9 vs art. 8 operator pay; decision-rule OC by effect location, attrition rule, gate sensitivity; EdU removed; many wording fixes.
Mini round M2	5 Opus + 2 Sonnet	All approve with changes; blocking: set-shift start-day allocation, time-limit and censoring rules, FX reserve vs ceiling, sponsor legal entity, route (a) vs (b)	Latin-rotation start days by block; protected ED budget, stage-failure and time-censoring rules (capped in the index, censored in Cox); home-cage interval on test day 1; fourth ELE operator (operator-hours 2,420; peak 67); satellite punch-first order; whole-pair euthanasia; analgesia regimen and humane endpoints; budget shown with FX reserve and 0a/0b itemised; LC 224/2025 in force; ICMS attestation corrected; AEP limits; art. 8 vs art. 9 pay; sponsor entity; state vs federal hosts; wording fixes.
Mini round M3	5 Opus + 2 Sonnet	(pending)	

12. Items the reviewers still do not like / remaining gaps

#	Gap	Kind
1	Bolus FGF17 is not the Iram infusion ($\approx 7\times$ less volume, pulsatile); the dose approximately matches Iram's reported $\approx 2.1\ \mu\text{g}$ total mass, but that figure rests on one reviewer's retrieval and exposure time is not matched; a null remains confounded with delivery.	Pumps in the larger study
2	Power: only very large effects detectable ($d\approx 1.6\text{--}1.8$ at 6–7 per arm); healthy rats near ceiling; the direction rule is lenient ($\approx 10\%$ false-go; $\approx 39\%$ go for enrichment alone), is driven by the spatial tasks ($\approx 28\%$ go if the effect is on set-shifting only) and cannot separate the drug from the environment because there is no drug-only arm.	Intrinsic to a mini pilot
3	The FGF17 gate uses one time point (30 min) and 4 rats per group; a true 1.5-fold engagement passes only about half the time, the heat-inactivated control and an assay CV $\leq 50\%$ are assumed, and the time course is an assumption; periventricular pERK shows bioactivity at the ventricular interface only. The Iram 2022 dose anchor (and so the single-pack purchase) must be confirmed from the Methods before ordering. The heat-inactivated control can still produce non-specific pERK.	Intrinsic; assay needed
4	7,8-DHF may be inert in vivo; 35 injections at the long end of precedent; the tolerability screen (2 rats per vehicle) only catches gross intolerance; DHF is not gated.	Declared
5	The ELE is solo, non-social and a weak dose; set-shifting is the only clean transfer readout; the arm cannot be fully blinded; parallel halves use opposite task orders (order effects are a fixed effect, arm $\times$ half not estimable).	Declared
6	Prices unverified (Brazilian per-diem not found); the base materials cost (11.2k) is close to the 12k ceiling and the pessimistic cases exceed it; personnel, overhead, equipment fees and a currency reserve are excluded and the all-in cost is probably USD 25–55k (unverified); the CNPq import exemption, the 2027 quota and the new CBS/IBS tax regime are uncertain (a refusal adds $\approx 1.0\text{--}1.5\text{k}$ and months).	Quotes and written confirmations
7	Host executability depends on a named roster (about 12–15 people at the busiest hours, about 6 on ordinary days), rooms and equipment, CEUA/CIAEP status, a controlled-drug route (MAPA/SIPEAGRO or ANVISA AEP), a legal entity that can contract for the individual sponsor, a legal instrument and a pre-funded first tranche, none of which are in hand.	Host to supply
8	Structure (Olig2/MBP histology) and synaptic readouts are not analysed in this study; tissue is only banked.	Larger study
9	Optogenetics (Stage 1b) and the human roadmap are aspirations without design detail or validated tools in the host lab.	Own review round
10	Literature checks used search snippets only; Iram's delivered dose was reported by one reviewer only; the FGF1 ICV pERK precedent is not referenced; human-trial and neurotrophin statements to be verified; Brazilian legal facts are from snippets and must be re-checked by the host.	Researcher/host to verify
11	The tolerability screen (2 rats per vehicle) cannot compare vehicles; the vehicle is a 35-day IP exposure in all arms; the periventricular gate uses one time point and 4 rats per group; the analysis model is ambitious for $\sim 18\text{--}21$ completers (a simpler fallback is pre-registered).	Declared

13. References and verification status

Checks were by search snippets only (PubMed, PMC, Crossref and publisher pages unreachable). “Verified” means consistent across indexed records.

#	Citation	Status
1	Iram T, Kern F, Kaur A, et al. Young CSF restores oligodendrogenesis and memory in aged mice via Fgf17. <i>Nature</i> 2022;605:509–515. doi:10.1038/s41586-022-04722-0 (an Author Correction exists)	Verified; delivered dose (25 µg/ml; 0.5 µl/h × 7 d ≈ 2.1 µg) reported by one reviewer, not independently confirmed
2	Boltaev U, et al. Multiplex quantitative assays indicate a need for reevaluating reported small-molecule TrkB agonists. <i>Sci Signal</i> 2017;10(493). doi:10.1126/scisignal.aal1670	Verified
3	Todd D, Gowers I, Dowler SJ, et al. A monoclonal antibody TrkB receptor agonist as a potential therapeutic for Huntington’s disease <i>PLoS One</i> 2014;9(2):e87923. doi:10.1371/journal.pone.0087923	Citation verified; DHF content via reviews
4	Pankiewicz P, et al. Do small molecules activate the TrkB receptor in the same manner as BDNF? Limitations of published TrkB low molecular agonists and screening for novel TrkB orthosteric agonists. <i>Pharmaceuticals</i> 2021;14(8):704. doi:10.3390/ph14080704	Verified (review)
5	Rawlings-Mortimer F, et al. 7,8-dihydroxyflavone enhances long-term spatial memory and alters brain volume in wildtype mice. <i>Front Syst Neurosci</i> 2023;17:1134594. doi:10.3389/fnsys.2023.1134594	Verified
6	van Praag H, Kempermann G, Gage FH. <i>Nat Rev Neurosci</i> 2000;1:191–198. doi:10.1038/35044558	Verified
7	Nithianantharajah J, Hannan AJ. <i>Nat Rev Neurosci</i> 2006;7:697–709. doi:10.1038/nrn1970	Verified
8	Galsworthy MJ, et al. <i>Behav Genet</i> 2005;35:675–692. doi:10.1007/s10519-005-3423-9	Verified
9	Matzel LD, Han YR, Grossman H, et al. <i>J Neurosci</i> 2003;23(16):6423–6433 (general learning ability in mice). doi:10.1523/JNEUROSCI.23-16-06423.2003	Verified
10	Gibson EM, Purger D, Mount CW, et al. Neuronal activity promotes oligodendrogenesis and adaptive myelination in the mammalian brain. <i>Science</i> 2014;344:1252304 (mouse optogenetic premotor-cortex stimulation)	Verified
11	Birrell JM, Brown VJ. Medial frontal cortex mediates perceptual attentional set shifting in the rat. <i>J Neurosci</i> 2000;20(11):4320–4324 (published protocols differ in stages; some add ID2, which is used here as a modification)	Verified
12	Kraemer HC, Mintz J, Noda A, Tinklenberg J, Yesavage JA. Caution regarding the use of pilot studies to guide power calculations for study proposals. <i>Arch Gen Psychiatry</i> 2006;63:484–489. doi:10.1001/archpsyc.63.5.484	Verified
13	Leon AC, Davis LL, Kraemer HC. The role and interpretation of pilot studies in clinical research. <i>J Psychiatr Res</i> 2011;45:626–629. doi:10.1016/j.jpsychires.2010.10.008	Verified
14	Percie du Sert N, et al. <i>PLoS Biol</i> 2020;18:e3000410. doi:10.1371/journal.pbio.3000410	Verified
15	Enrichment meta-analysis (rodent cognition $g \approx 0.44$, 95% CI 0.25–0.62): Fitzgerald, Univ. at Albany dissertation, a cross-species meta-analysis of environmental enrichment effects on cognition (scholarsarchive.library.albany.edu/etd/371); Léger M, et al. Environmental enrichment duration differentially affects behavior and neuroplasticity in adult mice. <i>Cereb Cortex</i> 2015;25(11):4048–4061. doi:10.1093/cercor/bhu119	Dissertation is not peer-reviewed; Léger verified
16	Fortin D, Rom E, Sun H, Yayon A, Bansal R. Distinct FGF/FGF receptor signaling pairs initiate diverse cellular responses in the oligodendrocyte lineage. <i>J Neurosci</i> 2005;25:7470–7479. doi:10.1523/JNEUROSCI.2120-05.2005	Verified
17	Nie S, et al. 7,8-DHF protects nigrostriatal dopaminergic neurons from rotenone-induced neurotoxicity in rodents. Rotenone model in 9-month-old rats, 7,8-DHF 5 mg/kg IP daily for 5 weeks. <i>Parkinsons Dis</i> 2019;2019:9193534. doi:10.1155/2019/9193534 (PMC6421741)	Verified
18	Gallego-Ortega A, et al. 7,8-Dihydroxyflavone maintains retinal functionality and protects various types of RGCs in adult rats with optic nerve transection. <i>Int J Mol Sci</i> 2021;22(21):11815. doi:10.3390/ijms222111815 (5 mg/kg IP daily; outcomes at 7, 14 and 21 days; the 1% DMSO detail was not confirmed)	Verified
19	Tait DS, Bowman EM, Neuwirth LS, Brown VJ. Assessment of intradimensional/extradimensional attentional set-shifting in rats. <i>Neurosci Biobehav Rev</i> 2018;89:72–84	Verified
20	Browne RH. On the use of a pilot sample for sample size determination. <i>Stat Med</i> 1995;14:1933–1940	Verified (title to check)
21	Gallagher M, Burwell R, Burchinal M. Severity of spatial learning impairment in aging: development of a learning index for performance in the Morris water maze. <i>Behav Neurosci</i> 1993;107:618–626. doi:10.1037/0735-7044.107.4.618	Verified
22	Rabinovitch MS, Rosvold HE. A closed field intelligence test for rats. <i>Can J Psychol</i> 1951;5:122–128 (Hebb-Williams maze)	Verified (issue number unconfirmed)
23	Zhang X, et al. Receptor specificity of the fibroblast growth factor family: the complete mammalian FGF family. <i>J Biol Chem</i> 2006;281:15694–15700 (FGF8 subfamily expected to act through FGFR3c/FGFR4; FGF17-specific profile not retrieved)	Real; FGF17 result unverified

Not found: any study of FGF17/FGF-family treatment improving learning in healthy young rodents; any study combining 7,8-DHF with enrichment; rat ICV FGF17 doses; independent confirmation of Iram 2022's delivered dose; Brazilian prices for Alzet pumps and recombinant FGF17; a peer-reviewed validation of a Hebb-Williams subset; a Brazilian per-diem housing cost. Brazilian regulatory and price sources checked by search (snippets only; to be re-checked by the host): Lei 11.794/2008 and Decreto 6.899/2009; CONCEA CIUCA manual and RN 49/2021; CNPq Portaria 2.248/2025; Unicamp Comunicado SEC PRP 001/2025; FUNPEC 2025 and FUJB 2026 quota notices; ANVISA RDC 172/2017; Convênio ICMS 93/98; Lei 10.973/2004, Lei 8.958/94; USP Rede de Biotérios and UFMG Biotério Central price lists.